

AN ADVISORY NOTE

ilé rollout plan phase one

Home healthcare and virtual consultations for older adults and dependents, targeted at local and diaspora families: objectives, service profile, go-to-market and a gated four-phase rollout.

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The wedge, and why it is the right one

ilé's long-term ambition is a trusted home-based and residential eldercare business. The right way to start is not with a building. It is with home healthcare services and virtual consultations, because that wedge is light on capital, quick to stand up, and it earns the two things a residential business will later depend on: a reputation for trustworthy care, and real data on who needs what. It also lets us serve the single most valuable early customer in this market, the Nigerian abroad with an ageing parent or a dependent relative at home, who has money, acute worry, and no one they fully trust to look in.

This plan sets out what the project must deliver, a detailed service profile centred on geriatric care but reaching the wider caring needs of dependents, who we serve and how we win them, the operating and technology model, a phased rollout, and how we will know it is working.

The strategy: create new market space, do not fight for the old one

The instinct in home care is to compete in the crowded, bloody water: undercut the informal caregiver market on price, or imitate the hospital at home. Both are losing games. The first is a race to the bottom on trust-poor, unaccountable labour. The second is expensive and tethered to a building. ilé's strategy is deliberately blue ocean: not to win the existing fight, but to open a market no one is serving, on terms no incumbent can match.

The move that makes it work is value innovation, breaking the usual trade-off between trust and cost. In this market, quality and trust normally come only at hospital prices, and low cost comes only with trust-poor informal labour. Technology collapses that trade-off. Virtual consultations, remote monitoring, the family dashboard and a shared workforce engine let ilé raise trust, clinical standard and visibility while lowering the cost of delivering them, because software and scheduled visits do what continuous in-person presence used to do, at a fraction of the cost.

The four moves that define the space are these.

Action	What ilé does
Eliminate	The opaque "drop a stranger at the house" model, the demand that the family self-coordinate the care, and the assumption that serious care needs a building
Reduce	Reliance on expensive round-the-clock in-person presence, the cost of specialist access, and the distance barrier that cuts a diaspora family out entirely
Raise	Trust and accountability, clinical governance and safety, and the calibre of the person who walks into the home, all far above the market norm
Create	Verifiable visibility of a loved one for a family member abroad, a single accountable care coordinator, planned and assessed care as the standard, and care that can be bought and paid for remotely, in foreign currency, from anywhere



The last row is the point. Visibility for a family abroad, and remotely purchased foreign-currency care, are not features the competition does poorly. They are dimensions of value that do not exist in this market today. That is where the blue ocean is.

The biggest prize is the noncustomer. ilé's growth does not depend on stealing clients from rivals. It depends on converting the vast group who use nothing formal today because nothing trustworthy exists: the diaspora son or daughter relying on a stretched relative, and the local family muddling through with help no one is checking. They are not in anyone's market yet. ilé creates that market, and defines it.

Seen against the alternatives, ilé's value curve is meant to diverge, not track.

Competing factor	Informal carers	Hospital-based care	ilé
Price to the family	Low	High	Middle
Trust and accountability	Low	Medium	High
Clinical governance and safety	Low	High	High
Care delivered in the home	High	Low	High
Specialist access	Low	Medium	High, via telehealth
Visibility for a family abroad	None	None	High, created
Diaspora reach and foreign-currency payment	None	Low	High, created

What this project must deliver

The strategic objective is simple to state: **make ilé the most trusted name in home-based care for older adults and dependents in Nigeria, monetised through both local families and the diaspora, and use that trust as the platform for the wider ilé healthcare business.**

Underneath that, ten specific objectives govern phase one.

1. **Stand up a licensed, clinically governed service** in Lagos, compliant with health-facility, telemedicine, nursing and data-protection requirements, before the first paying client.
2. **Define and hold a care standard** that is visibly better than the informal market: written care plans, trained caregivers, clinical oversight, and measurable outcomes.
3. **Prove the diaspora proposition**, that Nigerians abroad will pay, in foreign currency, for reliable care and honest visibility of their relative at home.
4. **Prove the local proposition**, that working professionals in Lagos will pay for care that takes the load of an ageing parent or a dependent off their shoulders.
5. **Validate the unit economics**, a repeatable, positive-margin care visit and a subscription that families renew.
6. **Build the workforce engine**, a vetted, trained, supervised bench of caregivers, nurses and allied professionals, sourced through CadreHealth.
7. **Build the technology platform**, the booking, care-record, telehealth, payment and family-visibility layer that Consult for Africa delivers.
8. **Establish clinical governance** under Dr Anele: protocols, incident management, and a safety culture from day one.



9. **Build the brand** as safe, warm and accountable, the opposite of the horror stories families fear.
10. **Prepare the ground for phase two**, multi-city expansion and, later, residential eldercare, without over-building before the model is proven.

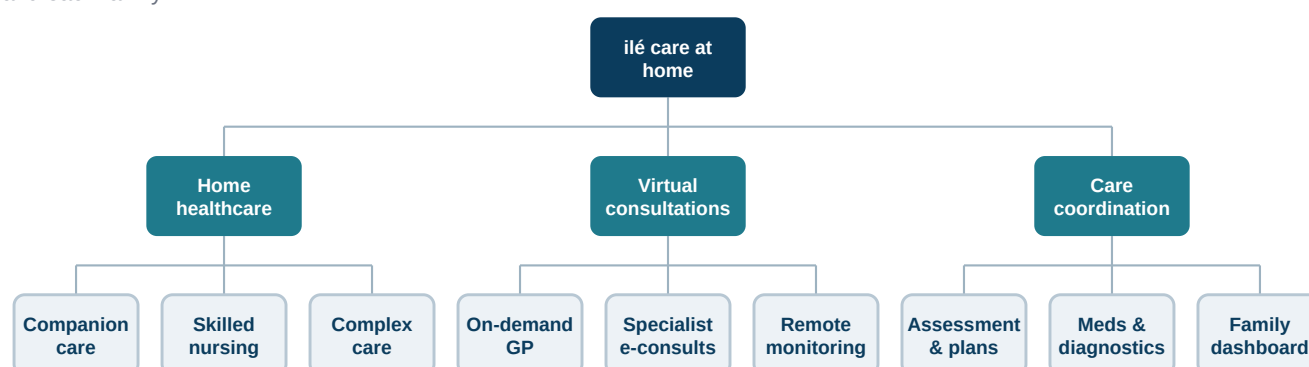
Each phase in the rollout below is gated: we do not scale spending until the objective for the phase before it is met.

The service profile

ilé's care model rests on one idea: care is planned, delivered by trained people, overseen by clinicians, and visible to the family. Every client begins with an assessment and a written care plan, and no one is left with an untrained stranger and no oversight. The service has three pillars.

FIGURE

The ilé service architecture. Three pillars, home healthcare, virtual consultations and care coordination, wrap around each client and each family.



Pillar one: home healthcare

Care delivered in the client's own home, organised in tiers so that a family pays for the level of care actually needed and can step up or down as the person's needs change.

Care tier	What it covers	Typical client
Companion and home support	Companionship, conversation, meal preparation, light housekeeping, medication reminders, mobility and walking support, errands, safety and welfare checks	An older adult who is largely independent but should not be alone all day
Personal care	Help with the activities of daily living: bathing, dressing, grooming, toileting and continence, feeding, transfers and repositioning	An older adult losing independence in daily living
Skilled nursing	Medication administration and injections, wound and pressure-sore care, catheter and stoma care, vital-sign monitoring, post-hospital discharge care, chronic-disease nursing	Someone with clinical needs who would otherwise be re-admitted or poorly managed at home
Specialist and complex care	Dementia and Alzheimer's care, stroke rehabilitation support, Parkinson's care, palliative and end-of-life care, care for the bedridden and for advanced chronic illness	The most vulnerable, needing trained, specialised, closely supervised care



Care tier	What it covers	Typical client
Allied health at home	Physiotherapy, occupational therapy, speech and language therapy, dietetics, podiatry, delivered as visits alongside the care plan	Anyone whose recovery or quality of life depends on rehabilitation and therapy

Geriatric care is the core, and it is more than nursing. The signature service is the **comprehensive geriatric assessment**: a structured review of an older person's medical, functional, cognitive, social and home-safety situation, from which the care plan is built. It is what separates planned care from a warm body sent to a house.

The caring needs we serve reach beyond the elderly, because families rarely have only one dependent and the same trusted service should carry all of them.

Beyond eldercare	What ilé provides
Post-surgical and convalescent recovery	Discharge-to-home nursing, wound care, mobility rebuilding, monitoring for complications
Chronic disease management	Structured home support for diabetes, hypertension, kidney disease, cancer supportive care, heart failure
Disability and special-needs support	Daily-living support and skilled care for adults and children living with disability
Maternal and newborn support	Postnatal recovery care, lactation support, newborn and premature-baby care at home
Long-term and bedridden care	Sustained personal and skilled care for those who cannot leave the bed, with pressure-injury prevention

Pillar two: virtual consultations

Telehealth is the second pillar, and it is what makes ilé affordable, fast and reachable, and what lets a doctor abroad or a specialist across town be part of the care without a journey.

- **On-demand primary care.** A general practitioner by video or phone, same day, for the everyday questions and worries that otherwise become an unnecessary hospital trip.
- **Scheduled specialist e-consultations.** Geriatrician, cardiologist, endocrinologist, neurologist, nephrologist, psychiatrist and palliative-care specialists, booked as video consultations, with the ilé nurse present at the bedside where needed.
- **Remote patient monitoring.** Connected devices for blood pressure, blood glucose, oxygen saturation and weight, feeding readings into the care record so a rising trend is caught before it becomes a crisis.
- **Care-coordination and family review calls.** Scheduled calls that bring the clinician, the caregiver and the family, including the family member abroad, into one conversation about how the person is doing and what changes next.
- **Second opinion and case review.** For diaspora families in particular, an ilé clinician who will review a diagnosis or a treatment plan and explain it plainly.
- **Mental health and caregiver support.** Counselling for clients and, importantly, for the family carers who are exhausted and rarely supported.

Pillar three: care coordination and enabling services

The wrap-around services that make the first two pillars safe, reliable and visible.



- **Assessment and personalised care plans**, reviewed and updated as needs change.
- **Home safety assessment and simple modifications**, to prevent the falls that put older people in hospital.
- **Medication management and pharmacy delivery**, so the right drugs arrive and are actually taken.
- **Home diagnostics**, sample collection for laboratory tests, ECG and basic investigations at home, with referral where imaging is needed.
- **Emergency escalation and ambulance coordination**, a clear, rehearsed path when something goes wrong.
- **The family dashboard**, the visit logs, photographs, care notes, clinician updates and billing that let a family, wherever they are, see that care is real and happening. This is the heart of the diaspora offer, described below.

Who we serve

Two customer groups, one service, deliberately designed so that the harder-to-serve, higher-paying diaspora segment is served to a standard that also delights local families.

Local families

Working professionals in Lagos, often the "sandwich generation" carrying children and an ageing parent at once, who love their parent and are stretched thin. They need competent, reliable care they can supervise, and relief from being the default nurse, driver and coordinator. The value proposition is time, peace of mind, and a standard of care they could not assemble themselves.

Diaspora families with local dependents

Nigerians in the United Kingdom, the United States, Canada and beyond, with an ageing parent or a dependent relative in Nigeria. This is the anchor segment for phase one. They have real income in strong currency, acute and constant worry, and, today, only informal and unaccountable options: a relative who is stretched, or hired help no one is checking. Their deepest fears are neglect, dishonesty and being kept in the dark. ilé is built to answer exactly those fears: trained and supervised carers, clinical oversight, and honest, frequent, verifiable visibility of their loved one. The value proposition, in one line, is **care for the people you left behind, delivered to a standard you can see and trust, paid for from wherever you are.**

How it works: the client journey

1. **First contact and needs conversation.** A family member, local or abroad, reaches ilé and describes the situation.
2. **Assessment.** A clinical assessment of the person, in the home for local clients, by video with a family member present for diaspora-initiated cases, leading to a written care plan and a clear quote.
3. **Matching and onboarding.** A caregiver or nurse of the right skill is matched, introduced to the family, and briefed on the care plan.
4. **Care delivery.** Visits happen on schedule, each one logged and verified, with clinical oversight in the background and telehealth on tap.
5. **Visibility and reporting.** The family sees every visit, note and update through the dashboard, and joins scheduled review calls.
6. **Review and adjustment.** The care plan is reviewed on a set cadence and stepped up or down as needs change, with billing that is transparent and predictable.



The diaspora trust engine

Winning the diaspora is a trust problem before it is a marketing problem, and trust is built through proof, not promises. ilé's answer is verifiable visibility:

- **Visit verification**, so a logged visit is a visit that genuinely happened, at the time claimed.
- **Photo, note and update after every visit**, timestamped, in the family's dashboard.
- **Clinician oversight the family can see**, so care is supervised, not freelance.
- **Scheduled review calls** that include the relative abroad as a full participant.
- **One accountable point of contact**, a named care coordinator, not a rotating call centre.
- **Foreign-currency billing** that is simple, predictable and receipted.

Get this right and the diaspora family stops worrying, tells every sibling and friend in the same position, and refers. Referral is the cheapest and strongest channel this business has.

The technology platform

Consult for Africa builds and runs the platform. Phase one needs a deliberately focused product, not everything at once:

- **Client and care records**, assessments, care plans, visit notes and clinical history in one place.
- **Scheduling and caregiver dispatch**, with a caregiver mobile app for check-in, check-out and visit logging.
- **The family dashboard**, the visibility layer described above, built for a user who may be five time zones away.
- **Telehealth**, video consultation with the clinician, with the nurse able to join from the bedside.
- **Payments**, local payment for local families and a clean foreign-currency path for the diaspora, with transparent receipts.
- **Remote-monitoring intake**, device readings flowing into the care record.

Everything else, richer analytics, deeper integrations, is phase two. Phase one ships the smallest platform that delivers safe, visible, billable care.

The build sequence

Consult for Africa builds in the order that lets demand and delivery come online together, not the order of an engineer's wish list.

- **Pre-launch**: the landing page and waiting-list capture, plus a simple back office to hold and segment the leads. This ships first, before the care platform, because it starts building demand while the rest is built.
- **Minimum viable platform**: client records, care plans, scheduling, the caregiver app, the family dashboard, telehealth and payments, the smallest set that delivers safe, visible, billable care to the pilot.
- **Scale build**: remote monitoring at volume, a deeper family dashboard, referral mechanics, diaspora-payment optimisation and the analytics to run the business.

Wherever a proven building block exists, video consultation, payments, messaging, ilé integrates it rather than rebuilding it, and Consult for Africa owns the roadmap and the pace.

The landing page: the front door and the first proof



Before ilé delivers a single visit, the landing page is the brand, and for a diaspora customer it is the first test of whether ilé can be trusted. It does three jobs at once: explain ilé in plain language, earn trust, and capture the waiting list.

Design principles. Warm, calm and credible, not clinical and cold. Real photography of real care. Plain language a worried adult child reads in one sitting. Mobile-first and fast, because much of the diaspora audience arrives on a phone. And trust worn on the surface: clinical oversight, vetted and trained carers, a named clinical lead in Dr Anele, and the promise that care is something the family can see.

What the page says, in order.

- A hero that speaks to the customer's heart: care for the people you left behind, delivered to a standard you can see and trust.
- How it works, in three simple steps: assessment, a matched carer, and visits you can watch happen.
- The services, framed as the care tiers and the wider dependent needs, so a visitor recognises their own situation.
- The trust engine: verified visits, photos and updates, clinician oversight, and one accountable coordinator.
- Who it is for, spoken to both audiences: the professional in Lagos, and the son or daughter abroad.
- Honest, human proof: the founding team and the clinical standard behind the promise.
- A clear, single call to action: join the waiting list.

What it captures. Enough to segment and prioritise, and no more: name, the best way to reach them including WhatsApp, whether they are local or abroad, who needs care and where, the relationship, and how soon. Every signup is then a known, segmented lead, not an anonymous email.

How we judge it. Signups and where they came from, the share that is diaspora, and the cost of each signup, so we learn which channels and which messages actually move this audience before we spend to scale them.

The waiting-list strategy

The waiting list is not a mailing list. It is how ilé manufactures validated demand, a warm pilot cohort and credibility, all before the service formally opens, and it is especially powerful for the diaspora, who want a way to signal their need now.

Why it matters.

- It proves demand is real, and shows where it concentrates, before we spend on delivery capacity.
- It gives phase one a running start: the pilot launches to real, waiting, pre-qualified families, not a cold market.
- It creates scarcity and momentum, a business people are queuing to join is a business worth joining.
- It teaches us the segments, urgency and geography of demand, which sharpens every later decision.

How it works.

- **Segment on capture**, local versus diaspora, care need, urgency and location, so we always know who is waiting and for what.
- **Make it a founding-family offer**, not a queue. Early joiners are offered founding-family status: priority onboarding, a free first assessment, and a pricing benefit honoured when they convert. This turns waiting into belonging.



- **Build a referral loop**, where a family moves up the list, or unlocks a benefit, by referring others in the same situation. The diaspora networks that share this need are dense, and referral is the cheapest demand ilé will ever get.
- **Nurture while they wait**, with genuinely useful content on caring for an ageing parent, and honest progress updates, so trust is being built before the first visit, not after.
- **Convert deliberately**. When the pilot opens, invite the highest-intent, best-fit families first, weighting toward diaspora-billed cases, so phase one begins with paying customers and the strongest possible proof.

What good looks like. A pre-launch waiting list of a meaningful size with a healthy diaspora share, a measurable referral rate inside it, and a clear, ranked shortlist of families ready to become the pilot cohort on day one. The illustrative targets below are a shape to aim at and refine, not a promise.

Illustrative targets. Phase zero, over roughly eight weeks: 300 to 500 waiting-list signups, at least 40% diaspora, a referral factor above 1.3, converting into a ranked shortlist of 30 to 50 pilot-ready families. Phase one then onboards 25 to 40 active clients, with at least half of the revenue diaspora-billed.

Workforce and quality

The service is only ever as good as the person who walks into the home, so the workforce is the product. ilé sources, vets and trains its caregivers, nurses and allied professionals through CadreHealth, the group's workforce engine, which is a genuine advantage over every informal competitor.

- **Sourcing and vetting**, background-checked, credential-verified, reference-checked care staff.
- **Training**, an ilé care standard covering eldercare, dementia care, personal care, safeguarding and conduct, before anyone is deployed.
- **Supervision**, clinical oversight, spot checks and a clear line of accountability.
- **Retention**, fair pay, respect and progression, because a stable, motivated carer is a safe carer, and the alternative is the churn that breaks trust.

Clinical governance sits with Dr Anele: the protocols, the incident-reporting and management system, the quality metrics and the safety culture. In a business caring for the vulnerable, safety is not a department, it is the licence to exist.

The commercial model

Revenue comes from care and from consultations, sold in the way each segment prefers to buy.

- **Pay-per-visit**, a priced nursing visit, caregiver hour, physiotherapy session or home diagnostic, for families who want to start small.
- **Monthly care subscriptions**, bundling a set number of hours or visits with virtual consultations, monitoring and reporting, at three levels, Essential, Enhanced and Comprehensive, matched to the care tiers.
- **Diaspora care bundles**, the subscription packaged and priced in pounds or dollars, with the visibility and reporting foregrounded and the foreign-currency convenience built in. The diaspora will pay a premium for reliability and peace of mind, and that premium is the margin that funds the quality.
- **Virtual consultations**, sold both pay-per-use and inside the subscriptions.



The figures below are illustrative, to show the shape of the ladder and to be calibrated in the pilot, not fixed prices. Pay-per-visit lets a family start small.

Service	Illustrative local price
Companion and home support	from N4,000 per hour
Personal care visit	from N6,000 per visit
Skilled nursing visit	from N18,000 per visit
Physiotherapy or allied-health session	from N18,000 per session
Home diagnostics, sample collection	from N6,000 plus test cost
Comprehensive geriatric assessment	from N50,000, one-off
Virtual GP consultation	from N12,000
Virtual specialist consultation	from N30,000

The subscriptions bundle care, consultations, monitoring and reporting into three levels matched to the care tiers. Diaspora bundles are the same care, priced at a premium that reflects the higher service level the segment demands and the strong-currency willingness to pay, and sold in pounds or dollars.

Tier	Who it suits	Local, from	Diaspora, from
Essential	Largely independent, needs presence, a check on wellbeing and light support	N180,000 per month	£160 / \$200 per month
Enhanced	Losing independence, daily personal care and chronic-condition support	N450,000 per month	£380 / \$480 per month
Comprehensive	Complex, skilled, near-continuous care with specialist access	N950,000 per month	£750 / \$950 per month

Every price, local or diaspora, must clear a positive contribution margin on the underlying visit, tested and confirmed in phase one before we scale. The diaspora premium is not a markup for its own sake, it is what funds the extra coordination, reporting and oversight that segment is paying to receive.

The rollout

Four phases, each gated on the one before, so spending follows proof.

FIGURE

The four phases. Each is gated on the one before, so spending follows proof, and the workforce grows ahead of demand rather than behind it.



Phase zero: foundations (roughly weeks 0 to 8)

Objective: be ready to deliver safe, licensed, visible care to a first client.

- Incorporate and license the entity, and settle clinical governance under Dr Anele.



- Design the care model, care plans, protocols and the ilé care standard.
- Ship the landing page and open the waiting list, so demand starts building while the rest is built.
- Build the minimum platform: records, scheduling, caregiver app, family dashboard, telehealth and payments.
- Recruit and train the first caregiver and nurse cohort through CadreHealth.
- Convert the waiting list into a pilot cohort of families, deliberately including diaspora-initiated cases.

Phase one: Lagos pilot (roughly months 2 to 5)

Objective: prove care quality, the diaspora proposition and the unit economics, at small scale.

- Serve an initial cohort of clients across the care tiers, with a meaningful share diaspora-billed.
- Hold the care standard, measure satisfaction and safety, and fix what the real world breaks.
- Confirm that families renew, that the diaspora pays in foreign currency, and that visits clear a positive margin.
- Produce an honest pilot read: what works, what to change, what to price.

Phase two: scale and open the diaspora channel (roughly months 5 to 10)

Objective: grow in Lagos and turn on diaspora acquisition deliberately.

- Expand the caregiver and clinical bench ahead of demand, never behind it.
- Market to the diaspora through the channels they actually gather in, community, faith, alumni and professional networks in the United Kingdom and United States, and lean on referral.
- Add remote monitoring at scale and deepen the platform.
- Hold quality as volume rises, which is the hardest and most important test.

Phase three: multi-city, and the ground for residential (roughly months 10 and beyond)

Objective: extend the proven model and prepare the next business.

- Open a second city, most likely Abuja, then Port Harcourt, using the proven playbook.
- Use the accumulated demand data and trust to lay the groundwork for residential eldercare homes, the capital-heavy phase that the group structure already anticipates ring-fencing.

Regulatory and compliance

Phase one operates inside a clear compliance perimeter, confirmed with counsel before launch.

- **Health-facility registration** for the home-care service with the relevant state regulator, HEFAMAA in Lagos.
- **Telemedicine compliance**, operating virtual consultations within the Medical and Dental Council's telemedicine framework, with properly licensed clinicians.
- **Professional licensing**, nurses and allied professionals registered with their councils, and clear scopes of practice.
- **Data protection**, registration and compliance under the Nigeria Data Protection Act, with health data treated as sensitive and the family dashboard built to protect it.
- **Safeguarding**, background checks, a conduct code and an incident regime appropriate to caring for vulnerable people.

Risks and how we manage them



Risk	Why it matters	How we manage it
A safety or neglect incident	The fastest way to destroy trust in eldercare	Clinical governance, training, supervision, incident management, and never deploying an unvetted carer
A breach of the diaspora's trust	Theft or dishonesty ends the referral engine	Verified visits, oversight, background checks, and a named accountable coordinator
Caregiver quality and reliability	The carer is the product	CadreHealth sourcing and training, fair pay, retention, and a bench so no visit is missed
Regulatory misstep	Licensing and telemedicine rules are real	Counsel-confirmed compliance before launch, licensed clinicians only
Foreign-currency and payment friction	The diaspora must be able to pay easily	A clean, transparent foreign-currency payment path from day one
Scaling faster than quality	Growth that outruns the care standard	Phase gates, and expanding the workforce ahead of demand, not behind it

How we will know it is working

- **Demand:** clients acquired, and the share that is diaspora-billed.
- **Quality and safety:** client and family satisfaction, incident rate, and care-plan adherence.
- **Engagement:** visits and consultations per client, and subscription renewal.
- **Workforce:** caregiver utilisation, retention and client ratings.
- **Economics:** revenue per client, contribution margin per visit, acquisition cost and lifetime value, and churn.
- **Trust, the leading indicator of all of it:** the referral rate, because a family that refers is a family that trusts.

Immediate next steps

- Confirm Lagos as the launch metro and agree the phase-zero timeline.
- Ship the landing page and open the waiting list first, so demand starts building immediately.
- Instruct counsel on licensing and the telemedicine and data-protection perimeter, alongside the founders' agreement already in motion.
- Scope and start the minimum platform build with Consult for Africa.
- Stand up the first caregiver and nurse cohort through CadreHealth, and settle clinical governance with Dr Anele.
- Convert the waiting list into the pilot cohort, including the first diaspora families, so phase one starts with real customers, not a cold start.

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